

Hand Hygiene

Policy Number	POL-IC-020	Owner / Department	Infection Prevention & Control
Effective Date	2023-06-01	Version	4.0
Last Revised	2026-01-28	Next Review	2028-01-28
Approved By	Infection Prevention & Control Committee	Applies To	All employees, licensed providers, students, volunteers, and contract staff in every patient-care and patient-adjacent area

1. Purpose

To prevent healthcare-associated infections by ensuring consistent, correct hand hygiene by all personnel, consistent with the CDC Guideline for Hand Hygiene in Health-Care Settings and the WHO *My 5 Moments for Hand Hygiene* framework.

2. Scope

Applies to everyone who enters a patient-care area, regardless of whether direct patient contact is anticipated. Hand hygiene is the single most effective measure to reduce transmission of pathogens and is non-negotiable at the indications defined below.

3. Definitions

- **Hand hygiene** — a general term for either washing with soap and water or applying an alcohol-based hand rub (ABHR).
- **ABHR** — alcohol-based hand rub containing 60–95% alcohol; the preferred method for routinely decontaminating hands when they are not visibly soiled.
- **Visibly soiled hands** — hands with visible dirt, blood, or body fluids; these require soap and water, not ABHR.
- **Point of care** — the place where the patient, the provider, and care involving contact come together; ABHR must be available at the point of care.

4. Policy and Procedure

Perform hand hygiene at each of the WHO Five Moments: (1) before touching a patient; (2) before a clean or aseptic procedure; (3) after body-fluid exposure risk; (4) after touching a patient; and (5) after touching the patient's surroundings.

1. Use soap and water when hands are visibly soiled, after using the restroom, before eating, and after known or suspected exposure to spore-forming organisms such as *Clostridioides difficile* or *Bacillus anthracis* (alcohol does not kill spores).
2. Otherwise, decontaminate hands with ABHR: apply a palmful, cover all surfaces of both hands, and rub until dry (about 20–30 seconds).

3. When washing, wet hands, apply soap, lather all surfaces for at least 20 seconds, rinse, and dry with a single-use towel; use the towel to turn off the faucet.
4. Perform hand hygiene before donning and after removing gloves — gloves are not a substitute for hand hygiene.

IMPORTANT — Hands must be decontaminated immediately before every clean or aseptic procedure — including insertion of any intravascular or urinary catheter — and immediately after any contact with blood or body fluids, even if gloves were worn. Do not proceed with an invasive procedure if hand hygiene has not been performed.

- Keep natural nail tips shorter than one-quarter inch. Personnel with direct contact with high-risk patients (e.g., ICU, OR) must not wear artificial nails or extenders.
- Do not wear rings or wrist jewelry during patient care that impede effective hand hygiene.
- Use facility-provided lotions only; petroleum-based products can degrade latex gloves.

5. Monitoring and Compliance

Program element	Standard
Direct observation	Trained observers audit a sampled number of hand hygiene opportunities per unit each month; the facility goal is $\geq 90\%$ compliance.
Product availability	ABHR dispensers at every point of care and room entrance; sinks stocked with soap and single-use towels at all times.
Feedback	Unit-level compliance reported to staff and leaders monthly; sustained shortfalls trigger a performance improvement plan.

6. Severity & Reporting

Hand hygiene lapses and related exposures are classified on the facility-wide scale in POL-RM-013. A lapse that contributes to a confirmed healthcare-associated infection or an MDRO transmission is reviewed as SEV-2 (major / temporary harm requiring intervention) and Risk Management and Infection Prevention must be notified within 4 hours. An observed breach in technique with no harm, or one intercepted before patient contact, is logged as SEV-3 or a Near Miss and reported to the Charge Nurse and Infection Prevention within 24 hours via the event report. When facts could support two levels, assign the higher level.

7. Documentation Requirements

- Monthly direct-observation audit results by unit, role, and the specific moment missed.
- Product-availability rounds and any stock-out remediation.
- For a lapse linked to an infection or transmission: the event report, assigned severity, and every notification made (who, when, by what method).

8. References

- CDC, Guideline for Hand Hygiene in Health-Care Settings, MMWR 2002;51(RR-16); CDC Core Infection Prevention and Control Practices.
- WHO, Guidelines on Hand Hygiene in Health Care (2009); *My 5 Moments for Hand Hygiene*.

- The Joint Commission, NPSG.07.01.01 (comply with hand hygiene guidelines); CMS Conditions of Participation, 42 CFR 482.42 (infection prevention and control).
- Related: POL-RM-013 (severity & reporting).